

RICHARDSON, GREGG W Admin Sex: **Male** DOB: **08/15/1969**

Ambulatory Summary

Summarization of Episode Note | 12/9/2019 to 12/9/2019

Source: Sports Medicine-Bountiful

Created: 04/29/2020

Demographics

Contact Information:

65 E BURTON LNKAYSVILLE, UT 840372829, US

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Mail: richardson.

Mail: RICHARDSON.GREGG@MSN.COM

Marital Status: Married

Religion: Church of Jesus Christ/Latter-Day Saints

Race: White, White

Previous Name(s): --

Ethnic Group: Not Hispanic or Latino

Language: English

ID: 540095541

Care Team

Type	Name	Represented Organization	Address	Phone
primary care physician	HORSLEY, DAVID G.	--	Work:390 N MAIN STBOUNTIFUL, UT 84010- , US	Work Tel: (801)397-6500

Relationships

No Data to Display

Document Details

Source Contact Info

280 N Main StBountiful, UT 84010-6136, US

Tel: (801)397-6750

Author Contact Info

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Recipient Contact Info

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Healthcare Professionals

No Data to Display

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Primary Encounter

Encounter Information

Registration Date: 12/9/2019

Discharge Date: 12/9/2019

Visit ID: --

Location Information

Sports Medicine-Bountiful

Work:280 N Main StBountiful, UT 84010-6136, US

Providers

Type	Name	Address	Phone
Attending	BELL, MD CHRISTOPHER C.	Work:280 N MAINBOUNTIFUL, UT 84010- , US	Work Tel: (801)397-6750

Encounter

FIN 1227472259 Date(s): 12/9/19 - 12/9/19

Sports Medicine-Bountiful 280 N Main St Bountiful, UT 84010-6136 US (801) 298-2495

Encounter Diagnosis

Knee pain, right (Discharge Diagnosis) - 12/9/19

Discharge Disposition: Home or Self Care

Attending Physician: BELL, MD, CHRISTOPHER C.

Reason for Visit

No data available for this section

Vital Signs

No data available for this section

Problem List

Condition	Effective Dates	Status	Health Status	Informant
Achilles tendinitis(Confirmed) ¹		Active		
Benign prostatic hypertrophy(Confirmed) ²		Active		
Exostoses(Confirmed) ³		Active		
Gastroesophageal reflux disease(Confirmed) ⁴		Active		
History of adenomatous polyp of colon(Confirmed)		Active		
Hyperglycemia(Confirmed)		Active		
Low back pain(Confirmed) ⁵		Active		
Plantar fasciitis(Confirmed) ⁶		Active		

¹ --Problem Type: Diagnosis

--Entered Date: 2011/12/19.00:00

² --Problem Type: Diagnosis

--Entered Date: 2010/06/04.00:00

³ --Problem Type: Diagnosis

--Entered Date: 2013/02/11.00:00

⁴ --Problem Type: Diagnosis

--Entered Date: 2004/09/23.00:00

⁵ --Problem Type: Finding

--Entered Date: 2010/06/04.00:00

⁶ --Problem Type: Diagnosis

--Entered Date: 2012/07/26.00:00

Allergies, Adverse Reactions, Alerts

Substance	Reaction	Severity	Status
sulfonamides	Urticaria (Hives) Rash	Moderate	Active

Medications

azithromycin (Azithromycin 5 Day Dose Pack 250 mg oral tablet) as directed on package labeling. Refills: 0. Ordering provider: HAYES, MD, HARLAND N.	
ciprofloxacin ophthalmic (ciprofloxacin 0.3% ophthalmic solution) 1-2 drops Both eyes every 4 hours while awake. Refills: 0. Ordering provider: HORSLEY, MD, DAVID GRANT	
diclofenac topical (diclofenac 1% topical gel) 2-4 gm Topical 4 times a day as needed as needed for pain. Refills: 5. Ordering provider: BELL, MD, CHRISTOPHER C.	BOWMAN'S PHARMACY 326 N Main St Kaysville, UT 840371106
fluticasone nasal (Flonase 50 mcg/inh nasal spray) 1 Sprays Nasal 2 times a day. Refills: 0. Ordering provider: FRANCOM, NP, SHAE	BOWMAN'S PHARMACY 326 N Main St Kaysville, UT 840371106
multivitamin	
multivitamin with minerals (PreserVision AREDS 2) Oral every day.	
pantoprazole (pantoprazole 20 mg oral delayed release tablet) 1 tabs Oral every day. Replaces omeprazole. Refills: 5. Ordering provider: CURRY, PA-C, CASSEYM.	BOWMAN'S PHARMACY 326 N Main St Kaysville, UT 840371106
pantoprazole (pantoprazole 20 mg oral delayed release tablet) 1 tabs Oral every day. Replaces omeprazole. Refills: 5. Ordering provider: CURRY, PA-C, CASSEYM.	BOWMAN'S PHARMACY 326 N Main St Kaysville, UT 840371106
sodium sulfate/potassium sulfate/magnesium sulfate (Suprep Bowel Prep Kit oral liquid) 177 Milliliters Oral once. as directed on the instruction packet. Refills: 0. Ordering provider: MERRILL, MD, JOSEPH T.	BOWMAN'S PHARMACY 326 N Main St Kaysville, UT 840371106

Results

No data available for this section

Immunizations

No data available for this section

Encounter Procedures

No data available for this section

Social History

Social History Type	Response
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Social History Type	Response
Smoking Status	Never smoker entered on: 1/13/20

Functional Status

No data available for this section

Reason for Referral

No data available for this section

Assessment and Plan

Extracted from:

Title: R Knee Pain	Author: BELL, MD, CHRISTOPHER C.	Date: 12/9/19
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RIGHT KNEE PAIN

SUBJECTIVE/HPI: States he noticed Reports there may some swelling in the knee. States there is pain after sitting for a few minutes and standing. Pain will go after a few steps. Noticed the pain when he's on the train and has o sit across from other passengers and his knee is bent at a sharp angle. Locates the pain mainly in the back of the knee. the pain at times seems to come across the front around the kneecap. States he does exercises, which includes cardio and weightlifting, to try to lose weight and keep good knee health.

Specific injury/MOI: NKi

Duration: ~ a few days ago

Location of pain/symptoms: R Posterior Knee

Character (sharp, dull): Pain, Aching, Stiffness

Severity of pain: 4/10

Constant or intermittent: Constant

Swelling: [Y]

Instability: [N]

Catching/locking: [N]

Popping/grinding: [N]

Worse with (stairs, squatting, etc.): Standing from Sitting, Going Down Stairs

Better with (treatment, rest, etc.): A Good Night's Sleep, Sitting with Leg Slightly Extended

Theater sign: Y

Night pain: [N]

Numbness/tingling: [N]

Fever: [N]

Primary sport(s) or recreational/exercise activity(-ies): _

Treatments attempted: IBU PRN; do they help?: N

Previously seen for this; if yes, when and with whom: N

Previous injury: N

Previous surgery on this knee: N

Previous injection (steroid or HA) and when: N

Previous imaging and when: N

Gen: INAD

HEENT: Normocephalic, no lesions, pupils equal, nares patent.

CV: Radial pulse 2+, cap refill brisk

PULM: No labored breathing, no pallor or cyanosis

SKIN: No lesions appreciated, skin warm, no erythema or ecchymosis

NEURO: Strength 5/5 RLE

R Knee

Inspection:

Neutral

neg edema

neg erythema

neg ecchymosis

1+ effusion

Patella:

Medial Facet TTP: [neg]

Lateral facet TTP [neg]

Patellar compression pain [neg]

ROM:

Flexion: 135

Extension: 0

Palpation:

Medial joint line TTP: [POS]

Lateral joint line TTP: [POS]

Quad Tendon TTP: [neg]

Patellar tendon TTP: [neg]

Hoffa's test: neg

Meniscus:

McMurray: neg

Steinman: neg

Ligament testing:

Lachman: no laxity

Valgus stress testing: neg

Varus stress testing: neg

Posterior drawer: neg

Pes Bursa:

TTP: neg

Pain with resisted knee flexion, "hacky sack," leg adduction: []

XR: 3-view R knee with WB PA view reveals no evidence of acute fracture or dislocation. Evidence of mild tricompartmental degenerative changes within the knee. Several loose bodies noted in the lateral compartment on the PA and sunrise views, not seen lateral view.

US reveals intact quad tendon, confirms small effusion.

A/P:

1. R KNEE PAIN: Most likely his pain is related to osteoarthritis, however there is possibility of a meniscus tear versus loose body versus osteochondral defect versus ligamentous injury. We discussed options in terms of further evaluation as well as management of the current symptoms. These include continued watchful waiting with over-the-counter pain relievers and anti-inflammatories as well as home versus formal physical therapy rehabilitation; further imaging with MRI to get a better look at the soft tissues and cartilage within the knee joint itself; knee joint steroid injection to help relieve the present symptoms. Reviewed these options as well as the pros and cons and risks and benefits at length, spending approximately 15 minutes with this. Following this discussion, the patient would like to proceed with:

[x] HEP ordered for knee, hip/core strengthening and stabilization

[x] Prescription for

- voltaren gel [x]

1. Knee pain, right